

The Special Supplemental Nutrition Program for Women, Infants, and Children (WIC) serves pregnant women, breastfeeding women (up to the child's first birthday), non-breastfeeding postpartum women (up to 6 months postpartum), infants (up to their first birthday), and children up to their fifth birthday. As such, it is the most important program providing nutritional support in the first 1000 days. In the United States, 53% of all infants younger than 1 year are served by WIC. According to the World Health Organization reports that 5-25% of preschool-age children suffer from minor brain dysfunction, including fine motor development disorders [2].

2. RESEARCH METHOD

This type of research is experimental quasy with the design of non-randomized control group pretest post-test design, in this study there were free variables and variables bound. This method of study uses the observation sheey using KPSP questionnaires (questionnaire pre-screening) [3]. Sampling techniques using purposive samples. The number of samples in this study of 42 respondents was divided into control groups and intervention groups.

3. RESULTS AND DISCUSSIONS

Table 1. Characteristics of Respondent

Characteristics	Frequency	Percentage
Age (Month)		
48	3	7,1%
60	18	42,9%
66	5	11,9%
72	16	38,1%
Gender		
Male	18	42,9%
Female	24	57,1%

Table 2. Effect of brain gymnastics therapy for the child's fine motor development (n=21)

Soft Motoric	Pre	Post	p-value
Mean $\pm$ SD	2,05 $\pm$ 0,498	2,38 $\pm$ 0,669	
Possibility (%)	2 (9.5%)	2 (9.5%)	0.020
Doubt (%)	16 (76.2%)	9 (42.9%)	
Development (%)	3 (14.3%)	10 (47.6%)	

This result showed that in the majority of intervention groups experienced the subduing of fine motor development according to age from 4 (19.0%) Respondents to 16 (76.2%) Respondents, while in the control group were increasing but not as much as the intervention group, it was seen from 3 (14.3%) Respondents to 10 (47.6%) which has a smooth motoic development according to age. This results in showing that brain gymnastics can affect the development of child fine motor, it can be proved in the intervention group which is mostly motor development according to age. The research results with the Mann Whitney statistical test obtained a P value of 0045. Decision making is done by looking at the degree of efficacy ($\alpha = 0.05$), i.e. when the P value of < 0.05 then H_0 is rejected. Current research because it is obtained by the value of < 0.05 , then there can be a meaningful difference between the development of the fine Motoik child 4-6 than in the control group and the intervention group seen from P value 0045 (< 0.05) so that it can be considered a therapeutic effect of brain gymnastics for fine motor development in children aged 4-6 in the sub-district of Surodadi subdistrict.

The increased physical complexity reflects the rapid rate of neuronal and glial growth and development within the brain. As these cells become more complex and interconnected, brain functional capacity increases. Some fundamental behavioral functions are already present at birth, implying that structural connectivity of the relevant brain circuits have begun. Examples of these early developing circuits include primary sensory systems such as hearing, touch and pain sensation, taste and, to a lesser extent, vision. More complex behaviors such as recognition memory, dependent in large part on hippocampal development, can even be demonstrated in the term and preterm neonate. Other brain functions that are being supported through this burst of circuit development and connectivity do not have obvious behavioral manifestations at birth. Despite the lack of behavioral manifestations, the neural circuits underlying more complex behaviors that will appear later in childhood are being constructed. For example, behaviors such as working memory, set-shifting, multi-tasking, and attention that are not present in the neonate nevertheless have their neural ontogenies prior to term birth.